Incident Report		
Reporter	Name: Cynde Sturm	Email:
Protocol	IRB No: IRB-XX-XXXX	IND No: XXXXXX
Site	xxxxxxx	
Investigator name	Principal Investigator, MD	
Date site became aware	XX/XX/202X	
Protocol deviation?	☐ Yes **X No**	
	If Yes, provide a detailed description below: N/A	
Adverse Event (AE)?	**X Yes** ☐ No	
	If Yes, provide a detailed description below: Participant is a XX-year-old, XXkg, White female with a past medical history of allergies, anxiety, arthritis, history of blood transfusion, cataracts, clotting disorder, chronic obstructive pulmonary disease (COPD), coronary artery disease, gastroesophageal reflux disease (GERD), hypothyroidism, kidney disease, and myocardial infarction, with a past surgical history of appendectomy, coronary artery bypass grafting (CABG), cholecystectomy, colon surgery, fracture surgery, hernia repair, and hysterectomy. Medications include calcitriol 0.25mg three times weekly, gabapentin 300mg three times daily, levothyroxine 0.2mg daily, atorvastatin 40mg daily, hydrocodone/acetaminophen 10/325mg three times daily, apixaban 5mg twice daily, losartan 25mg daily, budesonide/glycopyrrolate/formoterol inhaler twice daily, pantoprazole 40mg daily, metoprolol 25mg daily, ondansetron 8mg as needed, ketoconazole as needed, azelastine as needed, and cyclobenzaprine 5mg as needed; supplements and over-the-counter medications include a women's multivitamin, vitamin D3 5,000 IU daily, hair and nails supplement, coenzyme Q10 300mg daily, stool softener, psyllium fiber, polyethylene glycol, magnesium malate 1000mg daily, lubricating eye drops, loratadine 20mg daily, and famotidine 40mg daily. On XX/XX/202X, the participant reported severe diarrhea with abdominal cramping and fatigue beginning approximately five days after the third study drug dose. The participant reported five episodes of diarrhea requiring treatment with loperamide and a BRAT diet, with subsequent symptom improvement. The participant also reported an approximate 8-pound weight loss since starting study drug. During January and February 202X, the participant reported fluctuating	

symptoms, including alternating good and difficult days. Following dose escalation to 2mg, the participant noted worsening fatigue, post-exertional malaise, brain fog, neuropathy, constipation, and diarrhea that were most pronounced immediately following injections and improved later in the dosing interval.

On XX/XX/202X at 2 mg dosage, the participant reported worsening fatigue, brain fog, decreased energy and enthusiasm, increased sleeping and napping, increased heart rate, headache, increased pain/discomfort, labored breathing with activity, and upper respiratory symptoms following the most recent injection. The study coordinator contacted the participant and advised her to notify the study team if symptoms persisted or if she wished to decrease the study drug dose.

On XX/XX/202X, the participant reported elevated blood pressure and increased heart rate with activity after taking the first 3mg dose and planned to follow up with cardiology. The study coordinator requested an update following the cardiology evaluation. The participant subsequently reported that the cardiologist found no contraindication to continuing study participation.

On XX/XX/202X, following the first 3mg dose, the participant reported increased fatigue, brain fog, labored breathing, decreased energy and enthusiasm, excessive sleepiness, and irritability. The study coordinator advised the participant to seek medical evaluation if the labored breathing persisted or worsened and again offered study drug dose reduction.

During late April and early May 202X, the participant continued to report constipation alternating with diarrhea, persistent fatigue, and concern regarding minimal additional weight loss. The study coordinator discussed that weight loss is not always expected while receiving study drug and advised that daily polyethylene glycol (MiraLAX) was appropriate for management of constipation.

On XX/XX/202X, the participant contacted the study coordinator requesting a phone call to discuss ongoing side effects and study drug dosing. The participant reported persistent constipation but noted significant improvement in long COVID symptoms and expressed a desire to remain on study drug. During the call, the participant and study coordinator discussed reducing the dose to 2mg, with the possibility of a further reduction to 1mg depending on symptom response. The participant planned to follow up after an appointment with her treating physician on XX/XX/202X.

On XX/XX/202X, the participant reported ongoing constipation despite treatment with polyethylene glycol (MiraLAX) and docusate and noted an upcoming gastroenterology appointment. On XX/XX/202X, the participant reported severe fatigue that improved only two days each week, and the study coordinator again offered dose reduction because of persistent adverse events. On XX/XX/202X, the study coordinator reached out and

<table>
<tr><td></td><td colspan="3">offered to reduce dose, to which the participant agreed to decrease to 1mg.

At the subsequent study drug check-in, the participant reported improvement in fatigue following the dose reduction.

On XX/XX/202X, the participant reported hospitalization for severe epigastric abdominal pain with abdominal distention requiring evaluation after calling emergency medical services. CT of the abdomen and pelvis demonstrated duodenal distention without evidence of obstruction. The participant was treated with intravenous hydration, pain management, and antiemetics. Following hospitalization, the participant notified the study team of the decision to discontinue further study drug injections per the doctors' recommendations.</td></tr>
<tr><td>PID</td><td colspan="3">XXXXXX-XXXXXX</td></tr>
<tr><td rowspan="2">De-identified demographics</td><td>Age: XX</td><td>Weight: XXkg</td><td>Sex at birth: X Female ☐ Male</td></tr>
<tr><td colspan="3">Race/Ethnicity (select all that apply):
☐ American Indian or Alaska Native
☐ Middle Eastern or North African
☐ Asian
☐ Native Hawaiian or Pacific Islander
☐ Black or African American
X White
☐ Hispanic or Latino</td></tr>
<tr><td>Study drug (Tirzepatide/placebo) details:</td><td colspan="3">Specify details (e.g., start and last dose; therapy/usage started and stopped on; duration/frequency, date of dose reduction, etc):
Start date: XX/XX/202X

Study drug: Tirzepatide or placebo (blinded)
Dosing history:
● 1mg weekly: XX/XX/202X – XX/XX/202X (doses 1–8)
● 2mg weekly: XX/XX/202X – XX/XX/202X (doses 9–16)
● 3mg weekly: XX/XX/202X – XX/XX/202X (doses 17–21)
● 2mg weekly: XX/XX/202X – XX/XX/202X (doses 22–25)
● 1mg weekly: XX/XX/202X – XX/XX/202X (doses 26–29)</td></tr>
<tr><td rowspan="2">Serious Adverse Event (SAE)?</td><td colspan="3">X Yes ☐ No</td></tr>
<tr><td colspan="3">If Yes, indicate the SAE category below:
☐ Death
☐ Life-threatening
X Hospitalization
☐ Disability
☐ Congenital anomaly
☐ Other</td></tr>
</table>

Causality/relationship to study drug	☐ Not related ☐ Unlikely ☐ Possibly **X Probably** ☐ Definitely ☐ N/A
Rationale for causality assessment	The participant experienced progressive gastrointestinal symptoms, including constipation, diarrhea, fatigue, and abdominal discomfort, which increased in severity following dose escalation and prompted multiple contacts with the study team regarding study drug tolerability. Although symptoms improved after dose reduction, the participant subsequently developed severe epigastric abdominal pain requiring hospitalization. Imaging demonstrated duodenal distention without mechanical obstruction, and the treating physician included tirzepatide-related delayed gastric emptying in the differential diagnosis. Delayed gastric emptying is a known pharmacologic effect of tirzepatide and provides a biologically plausible mechanism for the event. While alternative etiologies, including ileus, partial small bowel obstruction, and duodenitis, were also considered, a causal relationship to the study drug cannot be excluded. Therefore, this event is assessed as **probably related** to the study drug.
Expected?	**X Yes** ☐ No
Date of onset	**X/X/202X**
Outcome	☐ Resolved **X Ongoing** ☐ Fatal ☐ Unknown ☐ N/A
Date of resolution	N/A
Study MD contact notes: Including relevant history/risks, diagnostics, concomitant medications	
Action taken with study drug	☐ None ☐ Dose reduced ☐ Dose increased ☐ Interrupted **X Discontinued** ☐ Other (specify)
Non-drug related actions taken (e.g., supportive tx for AE)	
Impact assessment	Does the incident pose an actual or potential risk to participant safety? **X Yes** ☐ No ☐ Unsure

	If Yes, provide a justification or explanation below: The participant required hospitalization for severe epigastric abdominal pain and abdominal distention. Diagnostic imaging demonstrated duodenal distention, and the participant required treatment and inpatient monitoring.
	Does the incident have the potential to substantially affect the completeness, accuracy, or integrity of the study data? ☐ Yes **X No** ☐ Unsure If Yes, provide a justification or explanation below:
Date reviewed by PI	
Date reported to DSMB	
Date reported to IRB (if SAE)	
Date reported to FDA (if IND safety report)	
Sponsor Comments and Recommendations	The participant was fully withdrawn and unblinded (upon participant's request) to aid in clinical management of their care.